CONFIDENTIAL Fax

FROM:

KAREN STEWART, MD
12952 Bandera Rd Ste. 105, HELOTES, TX 78023-4690
Phone: (210) 695-1900
Fax: (714) 276-2882

TO:

PAM HEALTH AT HOME
825 MAIN ST, BUDA, TX 78610
Phone: (512) 295-7000
Fax: (512) 295-7070

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[redacted]

Fax: 17142762882 **Pages:** 9

Phone: **Date:** 02-27-2026 5:10 PM

Re: Order(s) for Signature

Comments:

This fax contains orders for signature.
Please have the physician/clinician noted on this order.
Please sign, date, and return all pages to 1-512-295-7070.
Thank you!

PAM Health at Home – Texas
825 Main Street, Suite 110, Buda, TX 78610
Ph: 512-295-7000; Fax: 512-295-7070

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PAM Health at Home
825 Main St Ste 110, Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

ROC Order

Order No: R-26287 **Order Date:** 02/25/2026 10:00 AM

Patient

Vargas, Margarita (**MR#:** 24574)
5826 Magnes Ln
San Antonio, TX, 78227
(210) 387-7958
DOB: 04/24/1951

Physician

Karen Stewart MD
12952 Bandera Rd, STE 105
HELOTES, TX, 78023
(210) 695-1900 **Fax:** (714) 276-2882
NPI: 1295755981

Payer Source: PALMETTO GBA
Primary Diagnosis: I87.2 - Venous insufficiency (chronic) (peripheral)
Certification Period: 02/01/2026-04/01/2026

Requested Staff: Rogers, Jesse - RN

Order Description

SCHEDULE:
SN-SN Frequency: 1W6 Starting Week of 02/22/2026,
SN PRN Visits/Hours: 2 (Wound care for soilage/dislodgement)

Medications:
Lasix 20 mg tablet(Route=oral, Frequency=daily, Dosage=20mg, amount Of= 1.00)

Hydroxychloroquine 100 mg tablet(Route=oral, Frequency=daily, Dosage=100mg, amount Of= 2.00)

Sulfasalazine 500 mg tablet(Route=oral, Frequency=2 times a day, Dosage=500mg, amount Of= 2.00)

Ergocalciferol (vitamin D2) 1,250 mcg (50,000 unit) capsule(Route=oral, Frequency=weekly, Dosage=1250

Staff/Clinician Signature: Rogers, Jesse - RN **Date:** 02/25/2026 **Time:** 10:00 AM
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mcg, amount Of= 1.00)

Iron (ferrous sulfate) 325 mg (65 mg iron) tablet(Route=oral, Frequency=daily, Dosage=325mg, amount Of= 1.00)

Coreg 12.5 mg tablet(Route=oral, Frequency=2 times a day, Dosage=12.5mg, amount Of= 1.00)

Flomax 0.4 mg capsule(Route=oral, Frequency=daily, Dosage=0.4mg, amount Of= 1.00)

Eliquis 5 mg tablet(Route=oral, Frequency=2 times a day, Dosage=5mg, amount Of= 1.00)

Enalapril maleate 2.5 mg tablet(Route=oral, Frequency=2 times a day, Dosage=2.5mg, amount Of= 1.00)

Tylenol 325 mg tablet(Route=oral, Frequency=every 6 hours as needed for pain, Dosage=325mg, PRN/As Needed=Yes, amount Of= 2.00)

Amoxicillin 500 mg-potassium clavulanate 125 mg tablet(Route=oral, Frequency=2 times a day, Dosage=1)

Colchicine 0.6 mg tablet(Route=oral, Frequency=2 times a day, Dosage=1)

Interventions:

ANTICOAGULANT THERAPY
Clinician will assess/instruct patient/caregiver on anticoagulant therapy, to include diet, safety, reportable s/s. SN to instruct patient/caregiver on medication side effects to report to SN or Physician.

BEST PRACTICE FALL PREVENTION
Clinician will instruct patient /caregiver on fall prevention.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION
Clinician to perform drug regimen review/assessment to assess for potential clinically significant medications

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issues.
Clinician to assess/instruct on high risk and other medications to include effects, potential adverse effects, interactions and duplicate therapy, dose route and frequency.

BEST PRACTICE MONITOR AND MITIGATE PAIN
Clinician will instruct patient /caregiver on methods to mitigate pain to include medications and non-pharmacological interventions to decrease pain.

CARDIOVASCULAR: CHF
Clinician to assess/instruct patient/caregiver Symptoms in Heart failure indicated by clinical heart failure guidelines (including dyspnea, orthopnea, edema, or weight gain).

DIABETES
SN to instruct patient on diabetic diet as follows:
There are three main types of carbohydrates in food—starches, sugar, and fiber. As you'll see on the nutrition labels for the food you buy, the term "total carbohydrate" refers to all three of these types. The goal is to choose carbs that are nutrient-dense, which means they are rich in fiber, vitamins and minerals, and low in added sugars, sodium, and unhealthy fats.

WOUND OTHER (STASIS, SKIN TEARS,EDEMA,RASHES,ETC)
Clinician to assess NonPressure Chronic Ulcer wounds, instruct/perform wound care using aseptic technique as follows:
WCC Mondays and PAM HH Fri and PRN

R Medial Ankle: PRP in place secure with adaptic and steri-strips. Do not change anything under sterile strips. Cleanse with soap and water, cleanse with vashe. Apply alginate and ABD pad. Apply 50/50 mix Hydrocortisone cream and A&D ointment to right lower leg and foot. Then apply cast padding, coflex compression to RLE.

Right Plantar Foot Ball of Foot 2nd MT: Cleanse with soap and water, cleanse with Vashe. Apply Betadine to wound and peri wound, then 50/50 mix Hydrocortisone cream and A&D ointment to right lower leg and foot; cast padding, coflex compression to RLE.

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Left shin: Cleanse with soap and water, cleanse with Vashe. Apply Betadine to wound and peri wound, then 50/50 mix Hydrocortisone cream and A&D ointment Then apply cast padding, coflex compression to LLE.

HOSPITALIZATION RISK FACTORS
Clinician will instruct the patient and/or caregivers on the following unstable disease processes that may lead to unplanned hospitalizations: VENOUS INSUFFICIENCY, NON-PRESSURE ULCER- RLE, DM2, CKD, CHF, ANEMIA, GERD
Clinician will instruct patient and/or caregivers on polypharmacy, duplicate medications, adverse effects, et and compliance/understanding of medication regimen.
Clinician to instruct patient and/or caregivers on ways to avoid exhaustion and fatigue to include adequate sleep, nutrition, exercise, etc.
Clinician will instruct patient and/or caregivers on fall prevention to include clear pathways, adequate lighting, assistive devices, slow change in position, etc.

CARDIOVASCULAR - HTN
SN to instruct pt/cg on ways to manage HTN such as taking medications as prescribed, increasing physical activity, increasing foods/fluids high in potassium and calcium, managing stress, and avoiding high sodium foods, limit alcohol and monitor blood pressures at home.
SN to instruct patient/caregiver on Heart Healthy diet.

Goals:

ANTICOAGULANT THERAPY
LTG-Patient will be free of adverse s/s of anticoagulant therapy as evidenced by end of episode.

BEST PRACTICE FALL PREVENTION
LTG-Patient/caregiver will verbalize understanding of interventions to prevent falls by end of episode.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION
LTG-Patient /caregiver will be able to name s/s of adverse side effects by end of episode.

BEST PRACTICE MONITOR AND MITIGATE PAIN

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LTG-Patient/caregiver will demonstrate decreased pain with ambulation as evidenced by increased activity tolerance by end of episode.

CARDIOVASCULAR: CHF
LTG-Patient/caregiver will receive education on Symptoms indicative of heart failure and will be able to recall S/s of Heart failure and when to report by end of episode.

DIABETES
Hypo/Hyperglycemia sign and symptom free by end of episode.
Patient/caregiver will verbalize understanding of dietary requirements for ADA diet.

WOUND CARE
LTG-Pt wound status on BLE will exhibit objective signs of healing without complication at end of episode.

HOSPITALIZATION RISK FACTORS
Patient and/or caregivers will verbalize understanding of ways to mitigate risk of return to the hospital and unplanned ER use.

CARDIOVASCULAR - HTN
Patient/caregiver will be able to verbalize ways to manage HTN by end of episode.

DME:
Wound Supplies,Dressing Supplies,Walker,Other (specify): DM SUPPLIES, SHARPS CONTAINER

Supplies:
Probe Cover, Strl, For Digital Therm,Gloves,Latex Glove,Po-Free,Ns,Md,Alcohol Prep Pads, Medium,Other (specify): PPE MASK

Safety measures:
911 Emergency / Protocol,Assistance with IADLs,COVID PRECAUTIONS,DIABETIC PRECAUTIONS,Emergency Plan / Measures,Equipment Safety,Fall / Safety Precautions,Fall Precautions,Hazard-Free Environment,Infection Control Standard Measures and Precautions,Keep Pathways

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Clear,Keep Pathways Clear and Safe,Medication Safety / Storage,Safe in Movements,Safe Transfers,Safety in ADLs,Skin Care,Slow Position Change,Standard Precautions/Infection Control,Universal Precautions,Use of Assistive Devices

Nutritional Requirements:
No Added Salt,No Concentrated Sweets,Protein Hi Diet,Other (specify): HEART HEALTHY/RENAL DIET
Prognosis:
Fair
Mental Status:
Oriented
Functional Limitations:
Bowel/Bladder(incontinence),Ambulation,Other: Wound , diabetes, peripheral vascular disease
Activities Permitted:
Exercises Prescribed,Cane,Walker,Other: SBA
Diagnosis Codes:
I87.2 - Venous insufficiency (chronic) (peripheral)
E11.622 - Type 2 diabetes mellitus with other skin ulcer
L97.312 - Non-pressure chronic ulcer of right ankle with fat layer exposed
L97.322 - Non-pressure chronic ulcer of left ankle with fat layer exposed
J18.9 - Pneumonia, unspecified organism
J90 - Pleural effusion, not elsewhere classified
E11.22 - Type 2 diabetes mellitus with diabetic chronic kidney disease
I13.0 - Hypertensive heart and chronic kidney disease with heart failure and stage 1 through stage 4 chronic kidney disease, or unspecified chronic kidney disease
I50.40 - Unspecified combined systolic (congestive) and diastolic (congestive) heart failure
N18.30 - Chronic kidney disease, stage 3 unspecified
D63.1 - Anemia in chronic kidney disease
I89.0 - Lymphedema, not elsewhere classified
I31.39 - Other pericardial effusion (noninflammatory)
E11.51 - Type 2 diabetes mellitus with diabetic peripheral angiopathy without gangrene
D69.6 - Thrombocytopenia, unspecified
E89.2 - Postprocedural hypoparathyroidism

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E66.01 - Morbid (severe) obesity due to excess calories
E87.29 - Other acidosis
E83.52 - Hypercalcemia
K21.9 - Gastro-esophageal reflux disease without esophagitis
I25.2 - Old myocardial infarction
M06.9 - Rheumatoid arthritis, unspecified
Z91.81 - History of falling
Z28.311 - Partially vaccinated for COVID-19
Z68.43 - Body mass index [BMI] 50.0-59.9, adult
Z79.01 - Long term (current) use of anticoagulants

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).
All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.
Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.
May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation prn sob.

I certify in my estimation that the patient will need skilled care from home health for the next 60 days.
I attest to the following:
1. MARGARITA VARGAS is under my care.
2. I have received the following patient records from Be Healthy at Home:
a. Patient Summary Form
b. Hospital/LTC stay information (if applicable)
c. Home Health Plan of Care
d. And the signature below is my signature
3. Each of these patient records have been incorporated into my patient record.
4. These patients records were considered in advance of my certification of the patient's eligibility for home health services under Medicare.

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Physician Signature: _____ **Date:** _____

Electronically signed by KAREN STEWART, MD
on 03/01/2026 at 11:59:37 pm